

NOVACARE

INSURANCE

TO:	Chief Financial Officer, NovaCare Health Insurance
FROM:	Claims Analytics & Revenue Integrity Team
DATE:	April 20, 2026
RE:	URGENT — Claims Denial Crisis: \$7.22M Revenue at Risk
CLASSIFICATION:	CONFIDENTIAL — Executive Distribution Only

\$18M Total Billed	39.58% Denial Rate	\$7.22M Revenue Leakage	\$2.46M Recoverable Now
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EXECUTIVE SUMMARY

NovaCare is experiencing a systemic claims denial crisis that is directly costing the company revenue every day it goes unaddressed. Out of 6,150 claims filed and **\$18 million billed**, only 46.89% of claims were paid on first submission. A staggering **39.58% were denied** — generating **\$7.22 million in revenue leakage** and leaving an additional **\$2.46 million** in Pending and Appealed status at risk of expiring. This denial rate is 4 to 8 times higher than the industry average of 5–10%, and it has remained consistent across every single month of the calendar year, confirming this is a structural operational failure, not a seasonal fluctuation.

TOP 3 DENIAL DRIVERS

Driver 1 — Duplicate Claims (91.88% Denial Rate | \$1.4M Leakage)

A total of **480 duplicate claims** were submitted — and nearly every single one was rejected, producing a 91.88% denial rate. This is the highest denial rate of any category and represents the single most preventable source of revenue loss. Duplicate submissions are not a clinical issue; they are a process failure at the point of claim intake. No legitimate claim should ever be submitted twice.

Driver 2 — Patient Eligibility Failures (90.17% Denial Rate | \$0.9M Leakage)

Claims filed for patients who are not covered — either because their policy has lapsed, they are out-of-network, or their employer coverage has changed — are being denied at a **90.17% rate**. This means staff are regularly rendering services and billing for patients whose coverage has not been confirmed. With **\$0.9 million** lost to this cause alone, the cost of not checking is now quantifiable.

Driver 3 — Missing Authorization & Timely Filing (89.24% + 88.29% | \$1.7M Combined)

Pre-authorization is being skipped for services that require it, and completed claims are not being submitted within payer-mandated deadlines. Together, these two causes are responsible for **\$1.7 million in combined leakage**, at denial rates of 89.24% and 88.29% respectively. Both are entirely preventable with workflow enforcement and staff accountability.

REVENUE LEAKAGE QUANTIFICATION

Category	Amount Lost	Root Cause
Duplicate Claims	\$1.30M	Process failure — intake controls
Medical Necessity	\$1.04M	Documentation gaps
Patient Not Eligible	\$0.90M	No pre-service eligibility check
Timely Filing Error	\$0.90M	Missed submission deadlines
Coding Error	\$0.90M	Billing staff training deficit
Incomplete Documentation	\$0.80M	Incomplete clinical records
Missing Authorization	\$0.80M	Pre-auth not obtained
Unknown / Other	\$0.58M	Requires further investigation
TOTAL LEAKAGE	\$7.22M	40% of \$18M billed — actionable

The Emergency Department leads all departments in dollar leakage at **\$1.0M**. Insurance payers are responsible for the largest share of losses by payer type. Critically, **\$2.46M** of the total leakage sits in Pending and Appealed status — meaning the money has not yet been lost and is recoverable if pursued within 30 days.

3 SPECIFIC RECOMMENDATIONS

Recommendation 1 — Deploy Duplicate Detection at Point of Submission (Target: \$1.4M Recovery)

Require the claims management system to flag any submission that shares a patient ID, date of service, and procedure code with a previously filed claim. This check should prevent submission — not just flag it after the fact. With **480 duplicates** already on record at a **91.88% denial rate**, every new duplicate filed is a guaranteed loss. This is a 60-day technology change that pays for itself within the first week of operation.

Recommendation 2 — Mandate Real-Time Eligibility Checks at Scheduling (Target: \$0.9M Recovery)

Patient eligibility must be confirmed at the point of scheduling — not at billing. Every major Electronic Health Record system includes a real-time eligibility verification (REV) feature; most are simply not

activated or enforced. A **90.17% denial rate** on eligibility failures is evidence that patients are being seen with no valid coverage and the cost is being absorbed by NovaCare. This is a 30-day operational policy change, not a technology investment.

Recommendation 3 — Launch a 30-Day Recovery Sprint on \$2.46M in Pending & Appealed Claims

There is **\$2.46 million in Pending and Appealed claims** that is still recoverable today. These claims have already been filed and reviewed; all that remains is active follow-through before payer deadlines expire. Assign a dedicated team — even temporarily — to work this queue in the next 30 days. Prioritise the **Pediatrics (41.45% denial rate)** and **Emergency (40.71% denial rate)** departments, as they carry the highest volumes of unresolved denials.

WHAT WE WOULD INVESTIGATE NEXT

- **Dr. Yetunde Okafor — 71% Denial Rate on Viral Infection Cases.** This is a statistical outlier that warrants an immediate clinical and billing audit. A 30-claim focused review could determine whether this is a coding pattern, a documentation failure, or a payer-specific dispute — and resolve it within two weeks.
- **177 Claims with Date Errors.** This volume is too consistent to be random human error. We need to trace whether these originate from a specific EHR module, department, or intake workflow. If a single system configuration is generating date errors, one IT change closes all 177 simultaneously.
- **Pediatrics Department at 41.45% — Higher Than Emergency.** This is counterintuitive. Pediatric billing is typically predictable. The anomaly suggests either guardian eligibility lapses or referral authorization gaps unique to this department. Isolating the top 5 denial codes in Pediatrics alone could unlock an additional \$0.3–0.5M in recoverable revenue.
- **Government and Self-Pay Payer Mix at 40.9% and 40.6% Denial Rates.** The Self-Pay cohort may include patients who qualify for charity care or coverage assistance programs that NovaCare has not captured. A payer-mix audit could reveal whether revenue is being left uncollected from patients who do in fact have coverage.

This memo is based on a full analysis of NovaCare's claims dataset and the three-page Power BI dashboard reviewed by the analytics team. Every figure cited above is drawn directly from the dashboard data. Supporting charts and drill-down analysis are available in the accompanying slide deck and upon request.

Claims Analytics & Revenue Integrity Team

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